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December 21, 2024

By E-mail: bospositionstmts@mass.gov; mary.nicholls@mass.gov

Mary Nicholls, Investigator
c/o Cynthia Garcia, Office Support
The Commonwealth of Massachusetts
Commission Against Discrimination
18 Chestnut Street, Suite 520
Worcester, MA 01608

Re: Deila Vieira v. The Children's Hospital Corporation, Katherine Tecci
MCAD Docket No. 24NEM02630
EEOC / HUD No. 16C-2024-02621

Dear Ms. Nicholls:

Please accept the following as the position statement of Respondents, The Children's Hospital Corporation, also known as Boston Children's Hospital and Katherine Tecci (hereinafter, collectively, "Respondent" or "BCH") in response to the Charge of Discrimination ("Charge") filed by Complainant, Deila Vieira. Complainant, a former Patient Experience Representative in the Patient Services department of BCH, alleges that she was subjected to discrimination on the basis of her race, national origin and disabilities, and retaliated against for engaging in protected activity. Specifically, Complainant alleges that BCH failed to accommodate her need to take frequent bathroom breaks, counseled her for performance issues that she claims were not legitimate, and ultimately terminated her employment. She contends that the termination was based on her race, national origin and disabilities, as well as retaliatory motive.

BCH denies these allegations and submits that Complainant was not subjected to discrimination or retaliation of any kind. As a preliminary matter, some of the allegations in the Charge are time-barred, as Complainant sets forth various discrete allegations of purported mistreatment dating back to October 2021, nearly three years before she filed her Charge in August of 2024 and therefore well outside the statute of limitations. The timely allegations in the Charge are entirely without factual or legal merit. Complainant appears to contend that she was subjected to disability discrimination, but the record shows – and even her Charge shows – that she was fully accommodated with the frequent bathroom breaks that she requested. Moreover, while Complainant contends that she was subjected to unwarranted criticism of her performance, the documented record shows that she was treated fairly at all times, and that it was Complainant's long pattern of significant and sustained performance deficiencies which led her

to be placed on a performance improvement plan on two different occasions, and ultimately terminated when her performance issues continued.

Complainant's allegations of retaliation are equally without merit. Again, the record shows that she was fully accommodated, and the performance counseling and eventual termination of her employment were for documented and clear performance deficiencies, as discussed in detail below. Moreover, she contends that the purported mistreatment to which she was subjected had occurred from as far back as 2021, well before the protected activity which she claims led to her termination. Accordingly, her claim that she was treated adversely only after she engaged in protected activity by seeking accommodations is belied by her own allegations in the Charge.

Since Complainant has not and cannot offer any evidence to support her claims of discrimination or retaliation, the Commission should issue a finding of no probable cause and promptly dismiss the Charge.

I. FACTUAL BACKGROUND AND ANALYSIS

A. Boston Children's Hospital

BCH is a world-renowned acute care children's hospital located in the Longwood Medical Area in Boston. The hospital is home to the largest hospital-based pediatric research program in the world, treating pediatric patients throughout Massachusetts, the United States, and across the globe.

BCH proudly employs individuals from a wide range of backgrounds and places a strong focus on diversity, equity, and inclusion. BCH has no tolerance for any type of discrimination, retaliation or harassment in the workplace and has equal employment policies that are set forth in BCH's Equal Employment Opportunity policy. **See Exhibit A**. These policies apply to all BCH employees, and BCH is strongly committed to providing a workplace that is free of discrimination or harassment of any kind.

BCH's Code of Conduct sets forth additional rules and expectations with which all employees are expected to comply. Specifically, the Code of Conduct makes clear that communication, respect, excellence, accountability, teamwork and innovation are core values of the organization. The Code of Conduct can be found here: https://www.childrenshospital.org/sites/default/files/2022-03/2022-code-of-conduct.pdf

B. Complainant's Hire / PER Role

Complainant was hired in June 2021 as a Patient Experience Representative I ("PER") in BCH's Patient Services department. She was later advanced to a PER II role in July 2023. At the time of her hire, the person who would have been her direct manager was out on leave and never returned, so she reported for a brief period to her direct manager's manager, Katherine

Tecci, the Director of Perioperative Business Operations. In January 2022, Complainant had a new direct manager, as BCH hired a new Patient Services Administrator in Perioperative Care Services, Marie Ang. Ms. Ang was Complainant's direct manger for the rest of her tenure at BCH, although she was also indirectly supervised by Candace Mabry, who was responsible for supervising day-to-day workflow and the work of the PERs, but she did not have any direct reports. As Complainant notes in her Charge, Ms. Mabry is African American and Ms. Ang is Asian American.

As a PER, Complainant was responsible for providing support to the administrative operations of a clinical service (Complainant was assigned to Perioperative Services), and working to ensure the best possible patient experience by effectively coordinating services to patients and families. Complainant was also responsible for performing various administrative functions that required in-depth knowledge of BCH's programs and services and attention to detail.

Complainant was a "floater" PER, which means that she was assigned to various of twelve different workstations in the department, based on the needs of a given workstation on a given day. Each floater had this responsibility as well. The PER plays a critical role in ensuring that BCH's pediatric patients and their families receive excellent care and coordination of services, especially given the stresses of dealing with a child who is ill, and sometimes very seriously ill. Regular and reliable attendance is critical, as is the ability to perform tasks as they are designed, accurately and completely.

C. Complainant's Requests for Reasonable Accommodation

In her Charge, Complainant correctly notes that she disclosed to her supervisor, Marie Ang, that she suffered from irritable bowel syndrome ("IBS"), and that she presented a doctor's note seeking an accommodation that would allow her to take 3-4 bathroom breaks per day, for approximately ten minutes per break, upon a flareup of her IBS condition. (Charge, ¶34). As she acknowledges in her Charge, BCH granted this accommodation.

Complainant suggests that given this accommodation, it was improper for her managers to present her with performance counseling and discipline as a result of leaving her desk to take bathroom breaks. However, as discussed in detail below, the issue was not with Complainant taking bathroom breaks; the issue is that she would take lengthy breaks that went well beyond the reasonable amount of time needed for a bathroom break (sometimes coming back with a Starbucks drink, demonstrating that she had not merely used the break to attend to her IBS condition), and that she would not tell the nursing staff or other colleagues that she was leaving the desk, which would cause operational difficulties and confusion on a regular basis.

Complainant also contends that she requested, as an accommodation, to be placed in the Hale Building Preoperative Holding Unit and the Hale Post Anesthesia Care Unit ("PACU") because these units were slower paced than the busy day surgery department. (Charge, ¶¶37-40).

However, she did not request this as an accommodation to a disability – she simply requested slower-paced units because she preferred them, perhaps because she was having significant difficulty performing her job at an acceptable level. While Complainant alleges that Ms. Ang denied this request, this is simply untrue. A review of the schedules for the period in question demonstrates that Complainant was often assigned to the units in the Hale building which she preferred, and that she was only rarely assigned to the pre-operative location that she sought to avoid. Attached hereto are the scheduled for September through December 2023, demonstrating these facts. **See Exhibit B**.

D. Complainant's Serious Performance Deficiencies

Complainant demonstrated significant performance deficiencies throughout her tenure at BCH. As noted above, these were well documented and clearly failed to meet the standards expected of her as a PER at BCH. As early as August 2022, she had been issued a documented verbal counseling regarding concerns with her performance, attendance, and reliability. Specifically, concerns were raised about her tardiness and frequent periods of extended absence from her workstation, her excessive use of her phone and social media accounts at her workstation, her use of inappropriate tone and body language, her failure at times to engage respectfully and professionally with others. **See Exhibit C**.

1. October 2022 Written Warning

In October 2022, Complainant received an Employee Corrective Action Notice / Written Counseling which highlighted her failure to meet performance expectations in a number of critical respects. On September 21, 2022, she was working as a PER at the registration area when she failed to properly register the arrival of a patient for a surgical procedure. The nurses in the pre-operative holding area were looking for the patient, and they called Complainant at the registration desk to ask if the patient had arrived, and Complainant did not answer the phone. Complainant's failure to record the patient's arrival status, and her failure to be at the desk to answer the phone as is required of her, created delays and confusion among her colleagues and impacted patient care. Complainant was asked about this afterward and said it was a "one time mistake" and would not occur again. See Corrective Action Notice, dated October 7, 2022, attached hereto. **See Exhibit D.**

Only days later, on September 27, 2022, a colleague of Complainant complained to the manager that Complainant had taken an extended morning break (more than 15 minutes beyond the allotted time) and an extended lunch break (more than 30 minutes beyond the allotted time), which caused another staff member to have to miss her morning break. Two days later, on September 28, Complainant's supervisor, Candice Mabry, observed Complainant watching videos at the Registration desk and spoke to her about it. In response, Complainant said she watches the news during "downtime" and later emailed her manager with a screenshot showing that she was watching the news. Her manager reiterated to her that she should not be watching the news or other video content while she is at work unless it is work related. Complainant

questioned her manager, saying that that watching news during "downtime" is not doing any harm to patients or operations, and her manager reminded her that during downtime, there is a host of tasks that PERs can and should be performing, including preparing for the next day, helping out co-workers, and other tasks. *See Exhibit D*.

Complainant's performance issues continued only days later. On October 3, it was discovered that certain forms for the Post-Anesthesia Care Unit ("PACU") in the Hale Building were done incorrectly by Complainant the prior day; some slips were missing, other slips had incomplete procedures listed, and slips for the ICU rather than PACU were added in. A Charge Nurse in the PACU raised this issue with Complainant's manager, as this caused frustration, miscommunication, and delay when the PACU Charge Nurse was trying to get the slips ready for the morning's PACU announcements to staff. The following day, Complainant committed another error when she failed to perform any of the preliminary checks for the unit for which she was responsible, and instead she had done preliminary checks for another workstation for which another employee was responsible. Then, only one day later, Complainant forgot to give a printed schedule to the Director, which is part of her responsibility in that particular workstation. *See Exhibit D*.

The October 7, 2022 Corrective Action Notice / Written Warning issued to Complainant set forth a number of performance expectations going forward, including but not limited to that she would be expected to maintain good working relationships with her supervisor, manager, and other PERs in the Perioperative team, complete tasks and responsibilities assigned to her when covering a workstation, refrain from using social media while at her workstation, adhere to established break and lunch times, and adhere to established BCH processes and protocols. Complainant refused to sign this written warning. *See Exhibit D*.

2. November 2023 Written Warning

Complainant's performance issues continued in the summer and fall of 2023. On July 26, 2023, Complainant engaged in a safety violation that resulted in the filing of a SERS ("Safety Event Reporting System") report. **See Exhibit E**. She had failed to correctly admit a patient, resulting in two patients being placed in the same bedspace, a potential safety issue that was reported by the attending Anesthesiologist. Complainant was asked about this, and she replied that she couldn't remember the patient and couldn't remember placing two patients in the same bedspace, that she probably made a mistake and hit the "pending admission" key instead of hitting the key to "admit" the patient, and that she couldn't recall if the system issued a warning when she did so. Then, on September 19, 2023, Complainant's supervisors had to meet with her after nurses and other colleagues reported that she continued to step away from her desk without communicating with them, which creates additional potential safety and operational issues because as a PER, it is critical that she remain at the desk to coordinate the aspects of patient care for which she is responsible. These two items were the subject of verbal coaching, and they

were documented in a written warning issued to her on November 1, 2023, which was warranted after she had engaged in still more performance deficiencies. **See Exhibit F**.

In the weeks leading up to the November 1 written warning, Complainant was again the subject of complaints from nursing staff and other colleagues who reported that she would step away from her desk without informing anyone, leaving the desk without any PER coverage. On one occasion, there was an urgent case and phone calls that were forwarded to her desk, and she was gone for 20 minutes. At around the same time, a PER colleague reported that Complainant was unprofessional to him on the phone on a number of occasions. *See Exhibit F.*

On October 13, Complainant's failure to perform her job properly resulted in another SERS report being filed. She had incorrectly discharged a patient in the computer system while the patient was still in the operating room. This caused the Nurse Practitioner to be unable to find the patient and caused an extreme amount of confusion in what was already a very busy day in the PACU. This failure also impacted patient care by causing delays in getting the patient out of the operating room.

The November 1 written warning reiterated that Complainant had failed to meet BCH's Core Performance Standards for accountability, excellence, communication and teamwork. The warning set forth expectations that she would need to improve her attention to detail by being more careful when entering patient data into Epic (the electronic medical records system used by BCH), communicating her absence from the desk with her fellow PERs or the nursing team, maintaining professionalism when dealing with colleagues, and other expectations. The warning stated that immediate and sustained improvement in Complainant's performance is expected, and that if she fails to meet these standards, further disciplinary action in the form of a Performance Improvement Notice / Final Warning would result, or other discipline up to and including termination. *See Exhibit F.*

3. April 2024 Performance Improvement Notice / Final Warning

In early 2024, Complainant again demonstrated serious performance deficiencies and concerns that had a serious impact on unit operations and on patient care. On various dates in January, February and March 2024, she gave a patient an incorrect surgery and arrival time which caused a delay in the patient's surgery until the next day, disrupting operations and patient care. This resulted in the filing of yet another SERS Report. **See Exhibit G**. She also failed to answer phone calls placed to her desk when she was on duty, and she incorrectly entered nurse assignments and the transfer of bed spaces. She failed to perform a number of required tasks that impacted operations, and she made multiple additional errors in entering data into the system. She was repeatedly found using her personal cell phone rather than performing her work. She continued to leave her desk for extended periods without justification, outside of her break times, and without reporting her absence to her colleagues.

These incidents occurred on no fewer than *eleven different days* from January through March, all of which caused frustration, confusion, and an adverse impact on operations and patient care. In addition, these repeated performance issues clearly failed to meet the performance expectations set out in the October 13 written warning. As a result, Complainant was placed on a Performance Improvement Notice / Final Warning (known as a "PIN" at BCH). This document reiterated all of the expectations that had been set out previously, and it stated that immediate and sustained improvement in Complainant's performance was required, and that failure to do so may result in termination of her employment. The PIN also required Complainant to have regular 1:1 meetings with her manager and supervisor to discuss the status of her progress on each of the stated expectations. **See Exhibit H**.

D. The Decision to Terminate Complainant's Employment

Commented [GM1]: Question: The DTT memo says that Complainant received a 9/24/23 performance review with a 2 rating. I don't see that in her personnel file – the most recent review is dated 7/19/23. Do we have that 9/24/23 review, and if so, can you please send it to me?

In the weeks following the issuance of the PIN, Complainant's performance did not improve, and her performance deficiencies continued. On April 30, 2024, Complainant gave out incorrect patient information to the operating room team, reporting (incorrectly) that the patient was vomiting and had a high fever, causing her surgical procedure to be cancelled. The patient did not have these symptoms, and her procedure should not have been cancelled, so Complainant's careless mistake further compromised patient care and operational efficiency.

On May 6, 2024, Complainant was working at the Perioperative Care Coordination Clinic front desk, and a patient had to wait for an hour after Complainant told the patient that she had lost her chart. This caused a delay in patient care and an operational hardship because patients had to be shifted around. Complainant should have notified the charge nurse of this issue but failed to do so, even though the issue could have been rectified with the creation of a new file. Complainant also made another careless error when she marked two patients on the paper log schedule for the same nurse at the same time, causing further confusion and needless work to sort out the ensuing confusion. Also on the same day, Complainant allowed one of the patients leave the preoperative clinic without seeing the Anesthesia staff, and as a result there was no consent form signed, and there was no assessment conducted for this patient prior to surgery.

Complainant's consistent errors, lack of attention to detail, and other problems as documented throughout the progressive discipline process, and the fact that she had been coached repeatedly on these issues without significant improvement, were serious issues. In addition, Complainant's frequent failure to accept responsibility for these issues when they were discussed by her, instead choosing to deflect responsibility and blame others (much as she does in her Charge, notably), only compounded the problems. After this long history of concerns, and her failure to improve despite a long course of progressive discipline, her managers determined that they had no choice but to terminate her employment. These concerns are laid out in detail in the Decision to Terminate memo, dated May 21, 2024, which was prepared in advance of her termination. **See Exhibit I**. Complainant's employment was officially terminated on May 22, 2024.

II. ANALYSIS

Complainant's claims of discrimination, retaliation, and disability discrimination / failure to accommodate are entirely without merit, both as a matter of fact and law.

A. Complainant Was Not Subjected to Race or National Origin Discrimination

Complainant's Charge is devoid of any evidence that would support her claims of race or national origin. She appears to contend that she was unfairly disciplined because of her race (Black) and national origin (Cape Verde), but she has produced no evidence of that beyond noting that she happens to be Black and Cape Verdean. To the contrary, and as discussed in detail below, the performance counseling and discipline administered to Complainant, and the eventual termination of her employment, was based on entirely legitimate and non-discriminatory reasons. She demonstrated a long pattern of significant performance deficiencies, including frequently leaving her desk for extended periods without reporting her absence to nursing staff and other colleagues as required, making frequent careless mistakes, and continually failing to follow BCH policies and procedures and the requirements of her position as a PER. This led to frequent operational difficulties, delays in patient care, confusion and frustration by her colleagues, patients and their families, and potential safety issues. These issues have been well documented, as noted above. Complainant has produced no evidence, nor can she, that would indicate that she was singled out or that employees not of her protected class engaged in similar performance deficiencies and were not similarly counseled or disciplined.

In her Charge, Complainant refers to an incident in November 2021 where an individual named "Michelle," who was allegedly a Clinical Assistant, told her to "get out" of an office party, stating it was only for clinical assistants. (Charge, ¶18). As a preliminary matter, this alleged event occurred in November 2021 according to Complainant, which was nearly three years before she filed her Charge in August 2024. Accordingly, it is time-barred as a matter of law. Moreover, there is no evidence that this incident amounted to an act of discrimination. If a Clinical Assistant named Michelle made this statement to Complainant, BCH would not be legally responsible for the statement even if it were deemed to be discriminatory, as a Clinical Assistant would not be a supervisory employee, and Complainant never raised the issue before doing so in her Charge. Moreover, Complainant points out that "Michelle" was African-American, and one of the attendees at the gathering was also African-American (Charge, ¶19), which would refute her claim that she was uninvited to the party due to her race. Similarly, Complainant's claim that "Michelle" told her to leave a department Christmas party in December 2021 (Charge, ¶21) is also time-barred, and BCH cannot be responsible for an alleged comment by an unnamed non-supervisory employee. These claims simply do not withstand any scrutiny.

Commented [GM2]: Katherine – you'll recall that Delia claims that she reported these incidents to Robin Duggan and Dawn Brundage. You were going to follow up with them. Any update on that? I might be inclined not to respond this this part of the Charge specifically, but I'm interested to know what Robin and Dawn would say about this.

Commented [KT3R2]: I made outreach to Dawn and Robin and neither of them recalled this conversation or any instance of Deila reporting this to them. They noted that often Michelle will provider her own funds to buy items for a party (or collect from others to buy pizza) and perhaps this was the case in that those who were included were those who contributed to buying the food. But they do not recall this instance. I agree that it makes the most sense to leave out.

B. Complainant Was Fully Accommodated and Was Not Subjected to Discrimination on the Basis of a Disability

As discussed in detail above, Complainant's request for additional bathroom breaks to deal with her IBS condition was granted, demonstrating that BCH understands its obligations to reasonably accommodate its employees and that it met this obligation in Complainant's case. To the extent that Complainant was disciplined for the number and length of her breaks, this is because she would frequently take far longer than the allotted time period to use the bathroom, and would instead come back with a Starbucks beverage or other products that demonstrated that she did not merely take a bathroom break. In addition, she would violate Department procedures by failing to inform nursing staff and other colleagues that she was leaving her desk, which often impacted operational issues and led to unnecessary confusion and adverse impacts on the experience and care of patients and their families.

Complainant's allegation that BCH failed to grant her request to be assigned to less busy units in the Hale Building is without merit as a matter of fact and law. As noted above, she never made this request in connection with a disability – she simply expressed a preference to be assigned to the less-busy areas. Moreover, even if Complainant had tied this request to her IBS condition, this would not be a reasonable accommodation as a matter of fact or law. She was a PER floater, as noted above, which meant that she was required to be available to work at any of the 12 workstations in her area, depending on the needs of the patients and other operational demands. It would not have been possible, or consistent with the requirements of her job, to allow her to only work as a PER in a single area. To the extent that Complainant alleges that this is what she required, she will be unable to show that she was able to perform the essential functions of her job with or without a reasonable accommodation, and her claim should fail as a matter of law.

C. Complainant Was Not Subjected to Retaliation

Complainant contends that she was terminated in retaliation for requesting accommodations to her IBS, and complaining about allegedly discriminatory treatment. This claim is entirely without merit because it is belied by the long history of serious performance issues detailed herein. BCH gave Complainant every opportunity to improve her performance, through regular coaching and counseling, and then progressive discipline including verbal counseling, written warning, and a final written warning. Despite all of these efforts, Complainant's performance issues continued, leading to significant operational difficulty, confusion, and adverse impacts on patient care, patient experience, and safety.

As a pediatric children's hospital treating sick children, BCH's paramount concern is to provide excellent, safe, and responsive care to its patients and a fully supportive environment for families that are going though a challenging and often traumatic health situation. Complainant's

repeated performance issues compromised that effort, and BCH's decision to terminate her employment after a long period of continued issues was entirely legitimate and non-retaliatory.

D. Complainant's Claims Against Katherine Tecci are Without Merit

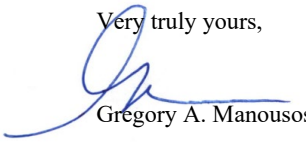
Complainant names Katherine Tecci as an individual Respondent in her Charge. (Notably, the fact that Complainant misspells Ms. Tecci's name as "Ms. Pecci" is a perfect example of her frequent mistakes and inattention to detail). However, Ms. Tecci played almost no role in any of the events and allegations giving rise to the Charge. Complainant mentions Ms. Tecci only briefly at the beginning of the Charge, noting that she serves as Complainant's supervisor at the time of her hire. However, as Complainant notes, only a few months after her hire, Marie Ang became her supervisor. Beyond the initial mention at the outset of the Charge, Ms. Tecci is never again mentioned as someone who had any role in the events giving rise to the Charge, beyond being present at the termination meeting. As discussed above, the decision to terminate Complainant's employment was based on entirely legitimate reasons. Accordingly, the claim against Ms. Tecci fails as a matter of fact and law.

III. CONCLUSION

Complainant has offered no evidence to support her claim that she was subjected to discrimination on the basis of race, national origin, or disability, or retaliation for engaging in protected activity. To the contrary, she was treated fairly and on the same basis as her peers, and all of the decisions at issue in this case were supported by legitimate, non-discriminatory reasons. For all of these reasons, BCH respectfully submits that the Charge should be promptly dismissed for lack of probable cause.

Should the Commission have any questions or need additional information to assist in its investigation, please do not hesitate to contact me.

Very truly yours,



Gregory A. Manousos

GAM/gmf
Enclosures
cc: Catherine West, Esq. (w/enc.)[via email: catherine@wyattlegalservices.com)

AFFIRMATION

I hereby state that with respect to the facts in the foregoing position statement of which I have personal knowledge, I affirm that such information is true and correct. With respect to the facts in the foregoing position statement of which I do not have direct personal knowledge, I affirm, to the best of my knowledge, that such information is true and correct.

Signed under the pains and penalties of perjury this ___ day of December, 2024.

_____
Scott Shannon
Senior Employee Relations Consultant
Boston Children's Hospital